

Clinical Diagnosis & History:

y/o female with two foci of carcinoma right breast 10:00/subareolar (FNA malignant) and 11:00/UOQ (core biopsy invasive ductal carcinoma). For right wide excision, SLNB, possible ALND.

Specimens Submitted:

- 1: SP: Sentinel node #1 level one right axilla (fs) (
- 2: SP: Sentinel node #2, level one right axilla (fs)
- 3: SP: Wide excision of tumor right breast 10:00 (

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TCGA-A0-A03P-01A-PR

Redacted



DIAGNOSIS:

- 1) SENTINEL LYMPH NODE #1, LEVEL I, RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE.
 - DEEPER LEVEL RECUTS AND SPECIAL STAINS HAVE BEEN ORDERED. THE RESULTS WILL BE REPORTED IN AN ADDENDUM.
- 2) SENTINEL LYMPH NODE #2, LEVEL I, RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE.
 - DEEPER LEVEL RECUTS AND SPECIAL STAINS HAVE BEEN ORDERED. THE RESULTS WILL BE REPORTED IN AN ADDENDUM.
- 3) BREAST, RIGHT, 10:00; WIDE EXCISION:
 - INVASIVE DUCTAL CARCINOMA, NOS TYPE, HISTOLOGIC GRADE III/III (SLIGHT OR NO TUBULE FORMATION), NUCLEAR GRADE III/III (MARKED VARIATION IN SIZE AND SHAPE), PRESENT AS THREE SEPARATE NODULES, RANGING IN SIZE FROM 1.0 CM UP TO 3.7 CM.
 - DUCTAL CARCINOMA IN SITU (DCIS) IS ALSO IDENTIFIED, SOLID TYPE WITH HIGH NUCLEAR GRADE AND MINIMAL NECROSIS.
 - THE DCIS CONSTITUTES <= 25% OF THE TOTAL TUMOR MASS, AND IS PRESENT ADMIXED WITH AND AWAY FROM THE INVASIVE COMPONENT.
 - NO CALCIFICATIONS ARE IDENTIFIED IN EITHER THE INVASIVE OR IN SITU CARCINOMA.
 - NO VASCULAR INVASION IS NOTED.
 - INVASIVE CARCINOMA IS 0.15 CM FROM THE NEAREST (SUPERIOR) MARGIN AND 0.4 CM FROM THE POSTERIOR MARGIN.
 - THE NON-NEOPLASTIC BREAST TISSUE IS UNREMARKABLE.
 - RESULTS OF SPECIAL STAINS (ER, PR, HER2-NEU) WILL BE REPORTED AS AN ADDENDUM.

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1CD-0-3

carcinoma, infiltrating duct, NOS 8500/3

Site: breast, NOS C50.9

lw
10/21/11

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	lw	10/21/11

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Special Studies:

Result

POSITIVE

Special Stain

AE1:AE3

Comment

NEG CONT
IMM RECUT
AE1:AE3
AE1:AE3
NEG CONT
IMM RECUT
ER-C
PR-C
HER2-C
NEG CONT
NEG-HER2
IMM RECUT

Gross Description:

1) The specimen is received fresh for frozen section diagnosis, labeled, "Sentinel node #1, level one, right axilla". It consists of a single lymph node, measuring 0.8 x 0.7 x 0.5 cm. The lymph node is bisected and entirely frozen.

Summary of Sections:

FSC frozen section control

2) The specimen is received fresh for frozen section diagnosis, labeled, "Sentinel node #2, level one, right axilla". It consists of a single lymph node, measuring 0.5 x 0.5 x 0.5 cm. Bisected and entirely submitted.

Summary of Sections:

FSC frozen section control

3) The specimen is received fresh, labeled, "Wide excision of tumor right breast 10:00 (short stitch superior, long stitch lateral)". It consists of an ovoid to irregular piece of yellow lobulated adipose tissue, measuring about 9.5 x 7.5 x 4.0 cm in greatest dimension. The specimen is oriented with a short stitch at the superior aspect and a long stitch at the lateral margin. The specimen is inked as follows: superior-blue,

** Continued on next page **

inferior-red, posterior-black, anterior-yellow. Serial sections of the breast reveal large centrally located 3.7 x 2.5 x 2.0 cm fairly well-circumscribed, tan lesion, located about 1.0 cm from the superior, inferior, and medial margins. The tumor is closest to the anterior and posterior margin and is at a distance of 0.5 cm from either of them. Cut surface of the tumor reveals a tan, slightly granular appearance. About 1.0 cm superior and lateral to the main mass are two smaller well-circumscribed nodules, the larger of which measures 2.2 x 1.3 x 1.0 cm. This fairly well-circumscribed nodule is < 0.2 cm from the superior margin and is located about 1.0 cm from the lateral and 0.5 cm from the posterior margin. About 0.5 cm lateral to this nodule is the smallest nodule measuring about 1.0 cm in diameter. The smallest nodule is located 0.2 cm from the superior and 0.5 cm from the lateral margin. The remaining portion of the breast parenchyma is composed of yellow lobulated adipose tissue. Representative sections of the specimen are submitted as follows.

Summary of Sections:

MM medial margin
 LM lateral margin
 T1 largest centrally located nodule
 T2 larger of the smaller nodules located lateral to the main mass
 T3 the smallest nodule
 R representative sections of the remaining breast tissue

Summary of Sections:

Part 1: SP: Sentinel node #1 level one right axilla (fs) (

Block	Sect.	Site	PCs
1		PSC	1

Part 2: SP: Sentinel node #2, level one right axilla (fs)

Block	Sect.	Site	PCs
1		PSC	1

Part 3: SP: Wide excision of tumor right breast 10:00

Block	Sect.	Site	PCs
3		LM	9
2		MM	6
2		R	3
5		T1	5
2		T2	2
3		T3	3

Procedures/Addenda:

Addendum

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Date Ordered:
Date Complete:
Date Reported:

Status: Signed Out
By:

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Addendum Diagnosis
ADDENDUM

SITE: SENTINEL LYMPH NODE, RIGHT AXILLA
PART #1, AND 2.

PART #1.

METASTATIC CARCINOMA IN THE FORM OF MICROSCOPIC CLUSTERS AND SINGLE CELLS
MEASURING LESS THAN 2MM (MICROMETASTASIS), IS IDENTIFIED ON CYTOKERATIN
IMMUNOHISTOCHEMICAL STAINS ONLY.

PART #2.

ONE ISOLATED CYTOKERATIN-POSITIVE CELL IS PRESENT. THESE CELLS ARE NOT
IDENTIFIED ON THE H&E STAINED SECTIONS.

Addendum

Date Ordered:
Date Complete:
Date Reported:

Status: Signed Out
By:

Addendum Diagnosis
ADDENDUM

SITE: RIGHT BREAST

ER: >95% OF NUCLEAR STAINING WITH STRONG INTENSITY.
PR: 40% OF NUCLEAR STAINING WITH MODERATE INTENSITY.
HER2/NEU (HERCEPT): NEGATIVE (STAINING INTENSITY OF 0).
CONTROLS ARE SATISFACTORY.

M.D.

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the
tissue sample examined at the time of the intraoperative

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consultation.

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- 1) FROZEN SECTION DIAGNOSIS: NO TUMOR IDENTIFIED.
PERMANENT DIAGNOSIS: SAME.
- 2) FROZEN SECTION DIAGNOSIS: NO TUMOR IDENTIFIED.
PERMANENT DIAGNOSIS: SAME.

** End of Report **